

DYNA CEPHALEXIN CAPSULE

	DESCRIPTION	CONTENT
DYNA CEPHALEXIN CAPSULE 250MG MAL07050014AZ	CAPSULE Capsule Colour : White/Scarlet Capsule Size : "1" Granules Colour : White	Each Capsule Contains: Cephalexin Monohydrate Equivalent To Anhydrous Cephalexin.....250 mg
DYNA CEPHALEXIN CAPSULE 500MG MAL07050015AZ	CAPSULE Capsule Colour : White/Scarlet Capsule Size : "0" Granules Colour : White	Each Capsule Contains: Cephalexin Monohydrate Equivalent To Anhydrous Cephalexin.....500 mg

PHARMACODYNAMICS:

Cephalexin is bactericidal and acts similarly to the penicillin by inhibiting synthesis of the bacterial cell wall. It is active against a wide range of bacteria including many Gram positive organisms and some Gram negative organism. Most Gram positive cocci are sensitive including both penicillinase- and nonpenicillinase-producing staphylococci, although methicillin-resistant *Staphylococcus aureus* are usually resistant; most streptococci are usually resistant. Some Gram positive anaerobes are also sensitive. Among the Gram negative organisms Cephalexin has activity against some Enterobacteriaceae including strains of *E.coli*, *Klebsiella pneumoniae*, *Proteus mirabilis*, and *Salmonella* and *Shigella* spp., it is also active against *H. influenzae* and *Neisseria* spp. Cephalexin is usually inactive against *Listeria monocytogenes*, *Bacteroides fragilis*, *Enterobacter* and *Pseudomonas* spp., indole-positive *Proteus*, *Serratia* spp., mycobacteria, mycoplasma, and fungi.

PHARMACOKINETICS:

Cephalexin is almost completely absorbed from the GI tract and produced peak plasma concentrations about 1 hour after administration. A dose of 100 mg produces a peak plasma concentration of about 18 mcg/ml; doubling the dose doubles the peak concentration. If Cephalexin is taken with food, absorption may be delayed, but the total amount absorbed is not appreciably altered. Up to 15% of a dose is bound to plasma proteins. The half-life has been reported to range from 0.5 to 2.0 hours and this increases with reduced renal function. Cephalexin is widely distributed in the body but does not enter the cerebrospinal fluid in significant quantities. It diffuses across the placenta and small quantities are found in the milk of nursing mothers. About 80% or more of a dose is excreted unchanged in the urine in the first 6 hours by glomerular filtration and tubular secretion; urinary concentrations greater than 1 mg/ml have been achieved after a dose of 500 mg. Probenecid delays urinary excretion and has been reported to increase biliary excretion. Therapeutically effective concentrations may be found in the bile.

INDICATIONS:

Treatment of the following infections when caused by susceptible strains of the designated microorganisms.

Respiratory Tract Infections caused by *S. pneumoniae* and group A - Haemolytic streptococci.

Otitis media due to *S. pneumoniae*, *H. influenzae*, staphylococci and/or *M. catarrhalis*.

Bone infection caused by staphylococci and/or *P. mirabilis*.

Genitourinary Tract Infections, including acute proctitis, caused by *E. coli*, *P. mirabilis* and *Klebsiella* spp.

Dental infection caused by staphylococci and/or streptococci.

RECOMMENDED DOSAGE:

Adult: The adult dosage ranges from 1 - 4 g daily in equally divided doses.

The usual adult dose: Oral, 250 - 500 mg every 6 hours.

For the following infections, a dosage of 500 mg may be administered every 12 hours: streptococcal pharyngitis, skin and skin structure infections and uncomplicated cystitis in patients over 15 years old.

Cystitis therapy should be continued for 7 - 14 days. For more severe infections or those caused by less susceptible organisms, larger doses may be needed.

Children: The usual recommended daily dose is 25 - 50 mg/kg in divided doses.

For streptococcal pharyngitis in patients over 1 year of age, mild, uncomplicated urinary tract infections, and for skin and skin structure infections, the total daily doses may be divided and administered every 12 hours. In the therapy of otitis media, clinical studies have shown that a dosage of 75 - 100 mg/kg/day in 4 divided doses is required. In the treatment of B-hemolytic streptococcal infections, a therapeutic dose of should be administered for at least 10 days.

CONTRAINDICATIONS:

Cephalexin is contraindicated in patients with known allergy to the Cephalosporin group of antibiotics.

WARNING AND PRECAUTIONS:

BEFORE CEPHALEXIN THERAPY IS INSTITUTED, CAREFUL INQUIRY SHOULD BE MADE CONCERNING PREVIOUS HYPERSENSITIVITY REACTIONS TO CEPHALOSPORINS AND PENICILLIN. CEPHALOSPORIN C DERIVATIVES SHOULD BE GIVEN CAUTIOUSLY TO PENICILLIN-SENSITIVE PATIENTS. SERIOUS ACUTE HYPERSENSITIVITY REACTIONS MAY REQUIRE EPINEPHRINE AND OTHER EMERGENCY MEASURES.

Reduced dosage is recommended in patients with severe renal impairment. The urine of patients taking Cephalexin may give a false positive reaction for glucose with copper-reduction reagents. Positive results to the Combs' test has been reported with Cephalexin and these can interfere with blood cross matching. Risk-benefit should be considered when the following medical problems exist: History of gastrointestinal disease, especially ulcerative colitis, regional enteritis or antibiotic-associated colitis.

Mean peak plasma concentrations of Cephalexin were reduced in healthy subjects when they also took cholestyramine.

Cephalexin was considered to be unsafe in patients with acute porphyria although there is conflicting experimental evidence on porphyrinogenicity.

Serious and occasionally fatal hypersensitivity reactions (including anaphylactoid and severe cutaneous adverse reactions) have been reported in patients receiving therapy with beta lactams. Before initiating therapy with DYNA CEPHALEXIN CAPSULE, careful inquiry should be made concerning previous hypersensitivity reactions to penicillins, cephalosporins, carbapenems or other beta-lactam agents. If an allergic reaction occurs, DYNA CEPHALEXIN CAPSULE must be discontinued immediately and appropriate alternative therapy instituted.

DRUG INTERACTIONS:

As with other beta-lactam drugs, renal excretion of Cephalexin is inhibited by probenecid. Probenecid decreases renal tubular secretion of Cephalexin, resulting in increased and prolonged Cephalexin serum concentrations, prolonged elimination half-life, and increased risk of toxicity.

PREGNANCY AND LACTATION:

Use in pregnancy & lactation: Laboratory experiments and clinical experiences show no evidence of teratogenicity, but it would be wise to proceed with caution during the early months of pregnancy, as with all drugs.

Pregnancy occurred in a woman taking an oral contraceptive after she had been given a course of Cephalexin by mouth for urinary-tract infection.

SIDE EFFECTS/ADVERSE REACTIONS:

Side effects of Cephalexin include nausea, vomiting, diarrhoea, hypoprothrombinemia (unusual bleeding or bruising), renal dysfunction (decrease in urine output or decrease in urine concentrating ability) and abdominal discomfort. Allergic reactions, such as skin rashes, urticaria, eosinophilia, angioedema, and anaphylaxis may occur, and rises in liver enzyme values have been noted. Neutropenia has been reported. Supra-infection with resistant microorganisms, particularly Candida, may follow treatment. There is the possibility of development of pseudomembranous colitis.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Symptoms of oral overdose may include nausea, vomiting, epigastric distress, diarrhoea and hematuria. Treatment of cephalosporin overdose should be symptomatic and supportive.

PACKING/PACK SIZE(S):

DYNA CEPHALEXIN CAPSULE 250MG - Blister pack of 100x10's.

DYNA CEPHALEXIN CAPSULE 500MG - Blister pack of 50x10's.

OTHER PREPARATIONS AVAILABLE:

DYNALEXIN DRY SYRUP 125MG - powder for reconstitution.

DYNALEXIN DRY SYRUP 250MG - powder for reconstitution.

**JAUHI DARIPADA KANAK-KANAK
KEEP OUT OF REACH OF CHILDREN**

MANUFACTURER/PRODUCT REGISTRATION HOLDER:

DYNAPHARM (M) SDN. BHD. 198001011897 (65683-V)

2497, Mk. 1, Lorong Perusahaan Baru 5,

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